SERENE

AESTHETICS, WELLNESS AND IV INFUSION CENTER

FREE GUIDE

Is It Perimenopause?

A Practical Guide for Women in Their 30s, 40s & 50s

SERENE MED SPA · PHYSICIAN-LED

Hudson, Ohio · Barboursville, West Virginia

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Educational guide · not medical advice

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Maybe your periods have started acting differently. Maybe you wake up at 3 a.m. for no reason, or you walk into a room and forget why. Maybe you just don't feel like yourself, and nobody can tell you why.

For many women, the answer is perimenopause: the years of hormone change leading up to the last period. It is a normal life stage, not a disease. But it can bring real symptoms that affect sleep, mood, work, relationships and health.

This guide explains what perimenopause is, the common and surprising signs, why it is so often missed, and what you can do about it. You'll also get a simple 4-week symptom tracker to bring

to your next visit, so you and your clinician can see the full picture.

About Serene Med Spa

Serene is a physician-led med spa with offices in Hudson, Ohio and Barboursville, West Virginia, led by Robin Arora, MD and Shweta Arora, MD. We plan every treatment around your goals and health history, and we'll tell you plainly what a treatment can and can't do.

PART 01

What perimenopause is (and isn't)

Perimenopause means "around menopause." During these years the ovaries don't simply wind down in a straight line. Instead, hormone levels rise and fall unevenly from month to month, and ovulation happens less reliably. Progesterone often drops first, while estrogen can swing from high to low.

Menopause itself is one moment in time: the point when you've gone 12 months in a row without a period (when there's no other cause, like pregnancy or a medication). The average age of menopause in the U.S. is about 51 to 52. Perimenopause is the stretch before that.

Most women notice changes in their 40s, but some start in their late 30s. Perimenopause often lasts about four years, though it can be shorter or much longer. Hot flashes and night sweats can continue for several years after the final period.

> **You can still get pregnant**
> Even with irregular cycles, pregnancy is possible in perimenopause. If you don't want to become pregnant, keep using birth control until a clinician confirms you've reached menopause.

PART 02

Common symptoms, and some surprising ones

No two women have the same experience. Some notice only a few small changes. Others feel like their body changed overnight. These are the symptoms women most often report:

- Period changes: cycles that are shorter, longer or skipped; heavier or lighter flow; spotting
- Hot flashes and night sweats (up to about 80% of women have them at some point)
- Trouble falling asleep or staying asleep, even without night sweats
- Mood changes: irritability, anxiety, low mood, tearfulness, or PMS that feels worse than before
- Brain fog: trouble focusing, slower thinking, losing words, forgetfulness
- Fatigue and lower energy
- Weight shifting toward the midsection, even without big changes in eating
- Lower sex drive, vaginal dryness, or discomfort with sex
- Less obvious signs: joint and muscle aches, heart racing or palpitations, new or changing headaches or migraines, breast tenderness, bloating, dry skin, adult acne, thinning hair, and more frequent bladder urges

> **Important**
> Every one of these symptoms can also have other causes, such as thyroid problems, low iron, sleep apnea, medications or stress. That's why a real evaluation matters, not just a guess.

PART 03

Why it gets missed, and why one blood test isn't enough

Perimenopause is common, yet many women go years without anyone connecting the dots. Symptoms often show up one at a time, so you might see one doctor for sleep, another for anxiety and another for joint pain, and nobody sees the pattern. It can also start earlier than people expect, and many clinicians received limited training in menopause care.

Another big reason: many women are told their labs are "normal" and stop asking.

A single hormone test usually can't give a clear yes or no. Hormone levels can swing widely from one week to the next, and even from one day to the next. A blood test only shows one snapshot. You could have a "normal" result on Tuesday and very different numbers two weeks later.

Hormonal birth control, a hormonal IUD or a past hysterectomy can also make hormone results harder to read.

For most women in their 40s and early 50s, perimenopause is identified by the overall picture: your age, how your cycles have changed, and your symptoms over time. That is why tracking matters so much.

Lab work is still useful, mainly to look for other conditions that cause similar symptoms or affect your treatment plan:

- Thyroid function (an under- or overactive thyroid can mimic perimenopause)
- Blood count and iron stores (heavy periods can lead to anemia and fatigue)
- Blood sugar and cholesterol (heart and metabolic risk rises around menopause)
- Pregnancy test when relevant
- Hormone levels in some cases, read in context rather than on their own

PART 04

What to track: your 4-week symptom tracker

A month of simple notes can tell your clinician more than any single lab. Use the tracker table in this guide (or your phone's notes app). Once a week, rate each symptom for the past 7 days: 0 = none, 1 = mild, 2 = moderate, 3 = severe. It takes about two minutes.

Bring the tracker to your visit, and keep tracking after you start any plan so you can see what's changing.

- Period start and end dates, flow (light, medium, heavy) and any spotting
- Hot flashes and night sweats: about how many per day or night
- Sleep: hours slept and how many times you woke up
- Mood, anxiety and irritability
- Brain fog and memory slips
- Energy, joint aches, headaches and palpitations
- Sex drive and vaginal comfort
- Possible triggers: alcohol, caffeine, spicy food, stress, a warm bedroom

> **Tip**
> Circle your top three symptoms, the ones that bother you most. Starting there helps you and your clinician focus the visit on what matters most to you.

PART 05

Lifestyle foundations that help

Lifestyle changes won't erase hormone shifts, but they can make a real difference in how you feel and protect your long-term health.

- Strength training at least 2 days a week to protect muscle and bone, which both start to decline faster in midlife
- About 150 minutes a week of moderate activity, like brisk walking, for heart health, mood and sleep
- Meals built around protein, vegetables, fiber and whole foods, which can help with midsection weight and blood sugar
- Limit alcohol: it often worsens hot flashes, night waking and mood
- A steady sleep routine with a cool, dark bedroom; layers you can remove at night
- Stress tools that fit your life: walks, breathing exercises, counseling, time with friends
- Don't smoke; quitting lowers heart, bone and cancer risks
- Cognitive behavioral therapy (CBT) has good evidence for easing how much hot flashes bother you, and CBT for insomnia (CBT-I) is a first-line treatment for trouble sleeping

> **Know your numbers**
> Body composition (muscle vs. fat) can change in midlife even when the scale doesn't. Both Serene offices offer a free InBody 580 body-composition scan starting October 21, 2026. It takes about a minute and gives you a printout to track over time.

PART 06

Treatment options, by category

There is no one-size-fits-all plan. The right approach depends on your symptoms, your stage of perimenopause, your health history, and your goals. Common categories include:

- Hormone therapy: estrogen (with a progestogen to protect the uterine lining if you have a uterus) is the most effective treatment for hot flashes and night sweats. It comes in pills, patches, gels, creams and, in the office, pellets. Clinicians generally aim for the lowest dose that controls symptoms and review the plan regularly.
- Cycle and bleeding management: some hormonal options can steady irregular or heavy periods, and some also provide birth control.
- Non-hormonal prescription options: several medications can reduce hot flashes for women who can't or prefer not to use hormones, including certain low-dose antidepressants and a newer class that targets the brain's temperature-control center.
- Local (vaginal) therapy: low-dose vaginal treatments and moisturizers can ease dryness and discomfort with sex. In-office intimate wellness treatments are also available for some women.
- Sleep and mood care: CBT-I, evaluation for sleep apnea, counseling, and medication when appropriate.
- Metabolic support: nutrition and strength-training guidance, and medical weight management when appropriate.

> **What's changed recently**
>
> In November 2025 the FDA announced it would remove broad boxed warnings about heart disease, breast cancer and dementia from many menopausal hormone therapy labels, and label updates followed in 2026. A boxed warning about uterine cancer remains for estrogen-alone products used by women who have a uterus. Hormone therapy still has risks and still isn't right for everyone. For healthy women who start within 10 years of menopause or before age 60, The Menopause Society says the benefits generally outweigh the risks for bothersome symptoms.

PART 07

Who may not be a candidate for hormone therapy

Your clinician will review your personal and family history before recommending hormone therapy. It may not be safe, or may need special care, if you have or have had:

- Breast cancer or another hormone-sensitive cancer
- Unexplained vaginal bleeding that hasn't been evaluated
- A blood clot in the leg or lung, or a known clotting disorder
- A stroke, heart attack or coronary heart disease
- Active liver disease

- Pregnancy, or a possible pregnancy
- Migraine with aura, smoking, or high blood pressure (these mainly affect which form and dose are safe)

> **If hormones aren't a fit**
>
> You still have options. Non-hormonal medications, CBT, sleep treatment and lifestyle changes can all help. Many women who can't take systemic hormones can still discuss local vaginal options with their care team.

PART 08

Telehealth or in person: how care works at Serene

Our physician-led team offers perimenopause care in person at our Hudson, Ohio and Barboursville, West Virginia offices, and by secure video through Serene Telehealth.

By video, Dr. Robin Arora can evaluate symptoms, review labs and prescribe non-controlled hormone therapy when appropriate for patients who are physically in Ohio, West Virginia, Kentucky or Florida at the time of the visit. A prescription is considered only after a live video evaluation, and only if it's appropriate for you. Follow-ups and messages happen in the same secure app.

Some care is in person only. Hormone pellets are placed in the office in Hudson or Barboursville. Testosterone and other controlled therapies are never prescribed by telehealth; they require an in-person visit. Telehealth is not for emergencies.

PART 09

Red flags: get prompt care

Irregular periods are expected in perimenopause, but some symptoms are not "just hormones." Contact a clinician promptly if you notice any of these:

- Very heavy bleeding, such as soaking a pad or tampon every hour for a few hours in a row, or passing large clots
- Periods that come closer than about every 3 weeks, or last much longer than usual
- Bleeding between periods or after sex
- Any bleeding after you've gone 12 months without a period
- Feeling dizzy, faint, short of breath or very tired with heavy periods (possible anemia)
- A new breast lump or changes in your breast skin or nipple
- Memory problems that steadily worsen, getting lost in familiar places, or trouble managing daily tasks

Call 911 or go to the ER for:

Chest pain or pressure; sudden shortness of breath; one-sided leg pain or swelling; sudden weakness, numbness, confusion or trouble speaking; a sudden severe headache; or seizures. If you have thoughts of harming yourself, call or text 988 any time, day or night.

Your 4-week symptom tracker

Rate each week: 0 none, 1 mild, 2 moderate, 3 severe	Week 1	Week 2	Week 3	Week 4
Hot flashes (daytime)	0 1 2 3	0 1 2 3	0 1 2 3	0 1 2 3
Night sweats	0 1 2 3	0 1 2 3	0 1 2 3	0 1 2 3
Trouble falling or staying asleep	0 1 2 3	0 1 2 3	0 1 2 3	0 1 2 3
Irritability or mood swings	0 1 2 3	0 1 2 3	0 1 2 3	0 1 2 3
Anxiety or feeling on edge	0 1 2 3	0 1 2 3	0 1 2 3	0 1 2 3
Low mood or tearfulness	0 1 2 3	0 1 2 3	0 1 2 3	0 1 2 3
Brain fog or forgetfulness	0 1 2 3	0 1 2 3	0 1 2 3	0 1 2 3
Fatigue or low energy	0 1 2 3	0 1 2 3	0 1 2 3	0 1 2 3
Joint or muscle aches	0 1 2 3	0 1 2 3	0 1 2 3	0 1 2 3
Headaches	0 1 2 3	0 1 2 3	0 1 2 3	0 1 2 3
Heart racing or palpitations	0 1 2 3	0 1 2 3	0 1 2 3	0 1 2 3
Low sex drive or vaginal dryness	0 1 2 3	0 1 2 3	0 1 2 3	0 1 2 3
Bleeding this week? (dates, flow, heaviest-day pad/tampon count)				
Possible triggers (alcohol, caffeine, stress, poor sleep)				

What it costs at Serene

Treatment	Hudson, OH	Barboursville, WV
Perimenopause or hormone consultation Ask when you book; the 20% new-client offer can apply.	See current pricing	See current pricing
Serene Telehealth program (video visits, follow-ups, secure messaging) For patients in OH, WV, KY or FL. Medication and labs are paid separately; insurance is not billed.	$149/month	$149/month
Hormone pellet therapy for women (in office) Starting price. Labs are separate. Final pricing confirmed at consultation.	$450	$405
Lab work Paid directly to the lab; reviewed with you at your visit.	Billed separately	Billed separately
FormaV intimate wellness treatment In office only. Results vary.	$500 per treatment; $1,350 for 3	$500 per treatment; $1,350 for 3
VTone pelvic floor treatment In office only. Results vary.	$350 per treatment; $1,500 for 6	$350 per treatment; $1,500 for 6
InBody 580 body-composition scan Starting October 21, 2026. About a minute, with a printout.	Free	Free

Prices as of October 2026; see serenemedspas.com for current pricing. New clients: 20% off your first visit — applies to any product or service; can't be combined with another discount.

Your consultation checklist

- [ ] Bring your completed 4-week symptom tracker and your top three symptoms
- [ ] Write down your period dates for the last 3 to 6 months, if you can
- [ ] List every medication and supplement you take, including birth control
- [ ] Know your family history of breast cancer, blood clots, stroke, heart disease and osteoporosis
- [ ] Bring any recent lab results, mammogram dates and Pap or cervical screening dates
- [ ] Ask: "Could anything besides perimenopause be causing my symptoms?"
- [ ] Ask: "Which labs do I need, and what will they tell us?"
- [ ] Ask: "Am I a candidate for hormone therapy? If not, what are my non-hormonal options?"
- [ ] Ask: "Do I still need birth control, and which options also help my symptoms?"
- [ ] Ask: "How will we measure whether my plan is working, and when do we follow up?"

Frequently asked questions

Can perimenopause start in my 30s?
Yes, for some women. Most notice changes in their 40s, but changes can begin in the late 30s. Symptoms at any age deserve an evaluation, because other conditions can look similar.

My hormone levels came back normal. Does that mean it's not perimenopause?
Not necessarily. Hormones swing from day to day in perimenopause, so one normal result doesn't rule it out. Your age, cycle changes and symptom pattern over time usually tell more.

Is hormone therapy safe?
For many healthy women under 60 or within 10 years of menopause, The Menopause Society says the benefits generally outweigh the risks for bothersome symptoms. It isn't right for everyone. Your clinician should review your personal and family history first.

Can I get hormone therapy by telehealth?
Yes, non-controlled hormone therapy can be prescribed by video when appropriate for patients physically in Ohio, West Virginia, Kentucky or Florida. Testosterone and pellets are in-person only, at our Hudson or Barboursville office.

Do I still need birth control?
If you don't want to get pregnant, yes. Pregnancy is still possible until menopause is confirmed. Some birth control options can also help with irregular or heavy bleeding.

Is brain fog a sign of dementia?
Usually not. Brain fog is common in perimenopause and often improves after the transition. But memory problems that steadily worsen, or trouble with everyday tasks, should be checked promptly.

What if I can't or don't want to take hormones?
There are effective non-hormonal options, including certain prescription medications, CBT, sleep treatment, local vaginal options and lifestyle changes. A good plan fits your health and preferences.

YOUR NEXT STEP

Ready when you are

- Fill in your 4-week symptom tracker, then book a visit with our physician-led team in Hudson (serenemedspas.com/hudson/) or Barboursville (serenemedspas.com/barboursville/).
- Prefer to text? Hudson: (330) 303-2031. Barboursville: (304) 396-0295.
- Live in Ohio, West Virginia, Kentucky or Florida? Start a video visit through Serene Telehealth at serenemedspas.com/telehealth/ for non-controlled hormone therapy and follow-up care.
- Stop by either office for a free InBody 580 body-composition scan (starting October 21, 2026) to get a baseline for muscle and body fat.
- New clients: 20% off your first visit — applies to any product or service; can't be combined with another discount.

Hudson, Ohio

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Text (330) 303-2031 · Call (330) 460-5915
serenemedspas.com/hudson

Barboursville, West Virginia

1 Chateau Grove Ln
Text (304) 396-0295 · Call (304) 520-0461
serenemedspas.com/barboursville

Scan a code to see pricing and book online.

Important

This guide is for general education only and is not medical advice. It does not diagnose, treat or replace care from a licensed clinician who knows your history. Treatment decisions, including hormone therapy, depend on an individual evaluation, and not everyone is a candidate. Results vary. Telehealth prescriptions are considered only after a live video evaluation and only when medically appropriate; testosterone, other controlled therapies and pellet therapy require an in-person visit. If you have a medical emergency, call 911. If you have thoughts of harming yourself, call or text 988.

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